

ZEJULA® Training Curriculum

Ovarian Cancer

Treatment Principles

Activity ID: PM-NZ-NRP-PPT-240003

Date of Approval: April 2024

Date of Expiry: April 2026

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Introduction



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Introduction and Learning Objectives

Welcome to the Ovarian Cancer Treatment Principles module of the ZEJULA® PRIMA Launch Training curriculum.

This module will introduce you to general principles and terminology used in the treatment of solid tumors, and present an overview of the treatment of ovarian cancer.

After completing this module you will be able to:

- Describe treatment goals and treatment approaches in patients with solid tumors
- Define first-line therapy, subsequent therapy, and maintenance therapy
- Distinguish between continuation and maintenance therapy
- Understand how the principles of solid tumor treatment apply in ovarian cancer
- Discuss unmet needs in ovarian cancer treatment
- Outline ovarian cancer post-treatment care

Principles of Solid Tumor Treatment



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Goals of Cancer Treatment

Cancer treatments can be classified by the goal of the treatment:^{1,2}



Curative treatment^{3,4}

- Achieve complete remission (disappearance of all signs and symptoms of cancer) and prevent cancer recurrence
- Long-term survival will not be limited



Life-prolonging treatment⁴

- Prolong survival, but without expectation of achieving a cure
- Long-term survival will be limited



Palliative treatment^{1,4}

- Reduce symptom burden and improve quality of life regardless of effect on survival
- Can be given alongside or independent of curative or life-prolonging treatments, not just near the end of life

1. Arnold RN. Palliative Care. In: Goldman L, Schafer AI, eds. *Goldman-Cecil Medicine*. 26th Ed. Philadelphia, PA: Elsevier Inc; 2020:10-15.
2. Ferris FD et al. *J Clin Oncol*. 2009;27:3052.
3. NCCN[®]. Patient and Caregiver Resources — Dictionary. Plymouth Meeting, PA; 2020.
4. Neugut AI et al. *Oncologist*. 2017;22:883.

Lines of Therapy

An overview of events in a typical course of therapy of a patient with a solid tumor is illustrated.^{1,2,3}

Newly diagnosed patients receive an initial treatment (first-line therapy).

If the patient experiences **clinical benefit**, treatment is typically followed by best **supportive care**.

When the first treatment stops working and disease **progression** occurs, a second set of treatments (second-line therapy or first recurrence therapy) can be initiated.

Further treatments (third-line and later-line therapy) can be initiated when the second treatment stops working.



[Click to learn about palliative care.](#)

Events during course of therapy^{1,2,3}

1. Freidlin B et al. *J Natl Cancer Inst.* 2015;107:djv225.
2. Grivas P et al. *Target Oncol.* 2019;14:505.
3. National Cancer Institute (NCI). *NCI Dictionary of Cancer Terms.* Bethesda, MD: U.S. National Institutes of Health (NIH); 2020.

Palliative Care



Palliative treatments can be given throughout the course of therapy, beginning at diagnosis and being delivered concurrently with disease-directed, life-prolonging therapies. Palliative treatments become the main focus of care when disease-directed therapies are no longer effective, appropriate, or desired.^{1,2}

1. Arnold RN. Palliative Care. In: Goldman L, Schafer AI, eds. *Goldman-Cecil Medicine*. 26th Ed. Philadelphia, PA: Elsevier Inc; 2020:10-15.
2. Dans M et al. NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). Palliative Care. V.1.2022. NCCN®. March 2022:1-109.

Glossary



Clinical benefit

Disease control or a response (complete or partial) after therapy, ie, the disease is not progressing.^{1,2}

Progression

Cancer that is growing, spreading, or getting worse; also called **progressive disease**.³

- Cancer returning after a period of time in which it could not be detected is called **recurrence**.
- Cancer that is getting worse after a period of improvement is called **relapsed**.

Supportive care

Care given to improve the quality of life of a patient by relieving symptoms of a disease or side effects caused by treatment, and providing psychological, social, and spiritual support. Supportive care does not involve active treatment of the cancer itself.³

1. American Cancer Society (ACS). *Managing Cancer as a Chronic Illness*. Atlanta, GA. January 2019.
2. Grivas P et al. *Target Oncol*. 2019;14:505.
3. NCI. *NCI Dictionary of Cancer Terms*. Bethesda, MD: NIH; 2020.

Lines of Therapy Icons

The terminology used by health-care providers to describe lines of therapy in different diseases can vary.

In ovarian cancer, lines of therapy are usually described as first line (or front line), second line, third line, and so forth.¹

In breast cancer, most patients receive curative therapy for early-stage disease as first-line therapy and, if their disease recurs as metastatic disease, then receive recurrence therapy for advanced disease. Therefore, lines of therapy in breast cancer are referred to as curative therapy for early disease, first-line (or front-line) therapy for advanced/recurrent disease, second-line therapy for advanced/recurrent disease, and so forth. Physicians who treat breast cancer patients may use this terminology.^{2,3}

Throughout the ZEJULA PRIMA Launch Training curriculum we will use standard ovarian cancer terminology. Different lines of therapy will be designated by these icons.



First-line therapy

- Also called front-line therapy



Second-line therapy

- Also called first-line recurrence therapy or front-line therapy for recurrent disease



Third-line therapy

- Also called second-line recurrence therapy



Later-line therapy



First-line maintenance therapy

- Also called switch maintenance therapy if a new agent, not part of the initial therapy regimen, is administered



Maintenance therapy in recurrent disease



First-line continuation therapy

- Also called continuation maintenance therapy



Continuation therapy in recurrent disease

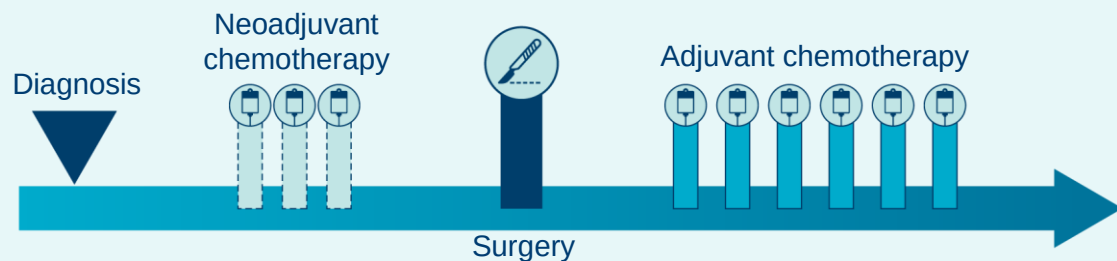
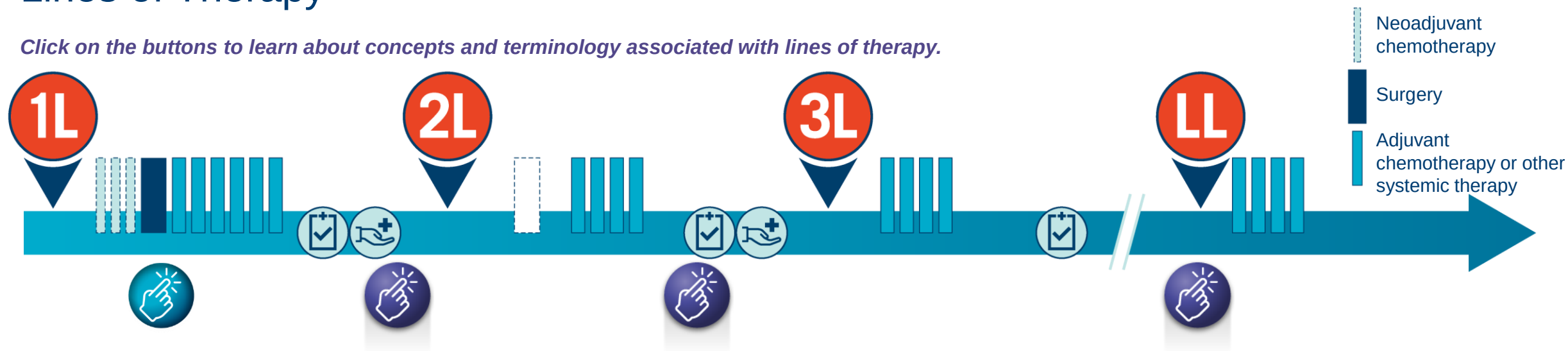
1. Ledermann JA et al. *Ann Oncol*. 2013;24 Suppl 6:vi24.

2. Reinert T et al. *Ther Adv Med Oncol*. 2015;7:304.

3. Henry NL et al. Cancer of the Breast. In: Niederhuber JE, Armitage JO, Doroshow JH, et al, eds. *Abeloff's Clinical Oncology*. 6th ed. Philadelphia, PA: Elsevier; 2020:1560-1603.

Lines of Therapy

Click on the buttons to learn about concepts and terminology associated with lines of therapy.



Example of first-line therapy^{1,2,3}

- First treatment given for previously untreated cancer^{4,5}
- Often part of a standard set of treatments, such as surgery following or followed by **chemotherapy** or radiation^{4,5}
- Sometimes called induction therapy or primary therapy^{4,5}
- In patients who undergo surgery or radiation as primary therapy, **neoadjuvant** and **adjuvant therapy** can be given³

1. Freidlin B et al. *J Natl Cancer Inst.* 2015;107:djv225.
 2. Grivas P et al. *Target Oncol.* 2019;14:505.
 3. NCI. *NCI Dictionary of Cancer Terms.* Bethesda, MD: NIH; 2020.

4. Sheard DA et al. NCCN Guidelines for Patients (NCCN Guidelines®). Ovarian Cancer Epithelial Ovarian Cancer. V.1.2021. NCCN®. 2021:1-72.
 5. West HJ et al. *JAMA Oncol.* 2015;1:698.

Glossary



Adjuvant therapy

Additional cancer treatment, such as chemotherapy or radiotherapy, given after primary treatment to lower the risk that that cancer will come back.¹

Chemotherapy

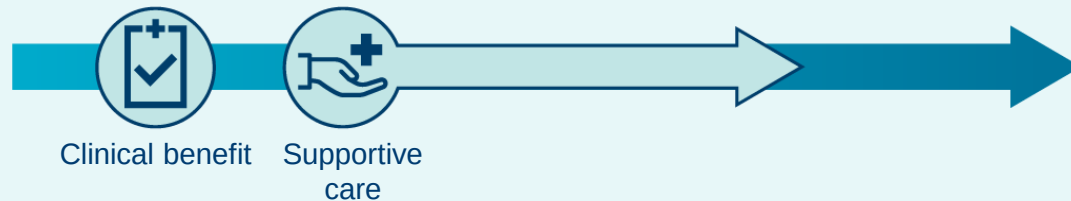
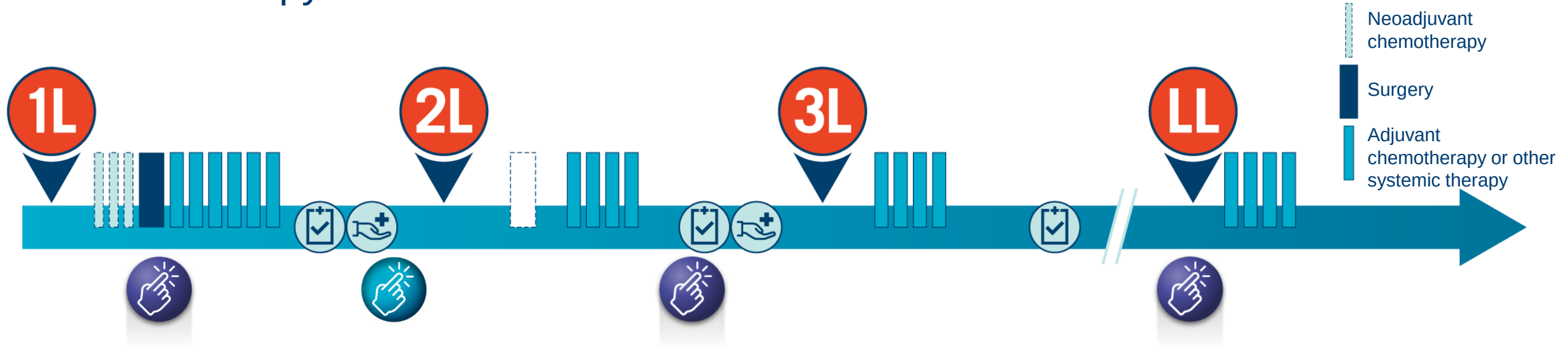
Treatment that uses drugs to stop the growth of cancer cells, either by killing the cells or by stopping them from dividing. It may be given alone or with other treatments, such as surgery, radiation therapy, or biologic therapy.¹

Neoadjuvant therapy

Therapy given before primary treatment to shrink the tumor before the main treatment.¹ Reducing the size of the tumor prior to surgery can improve the outcome of the surgery.²

1. NCI. *NCI Dictionary of Cancer Terms*. Bethesda, MD: NIH; 2020.
2. Rose PG, Uyar DS. Ovarian Cancer. In: Bieber EJ, Sanfilippo JS, Horowitz IR, et al, eds. *Clinical Gynecology*. 2nd ed. Cambridge, UK: Cambridge University Press; 2015:790-812.

Lines of Therapy



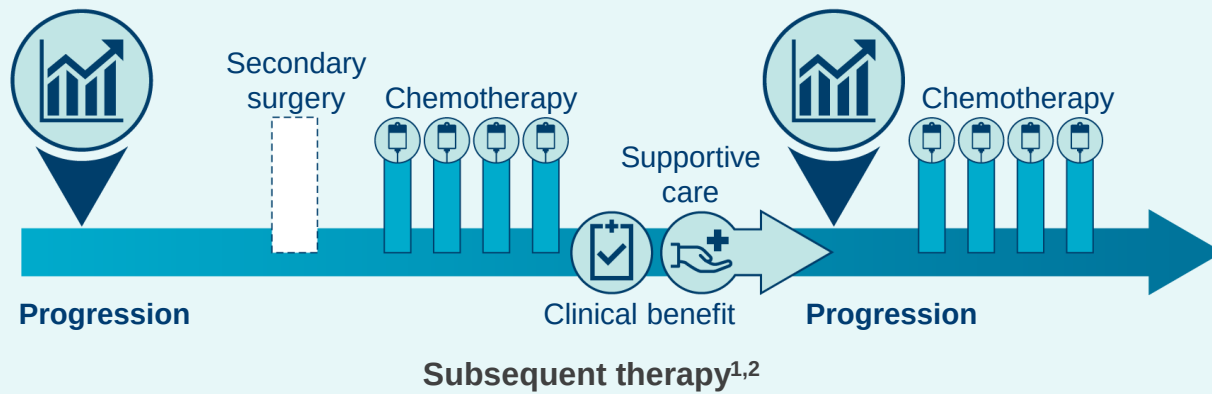
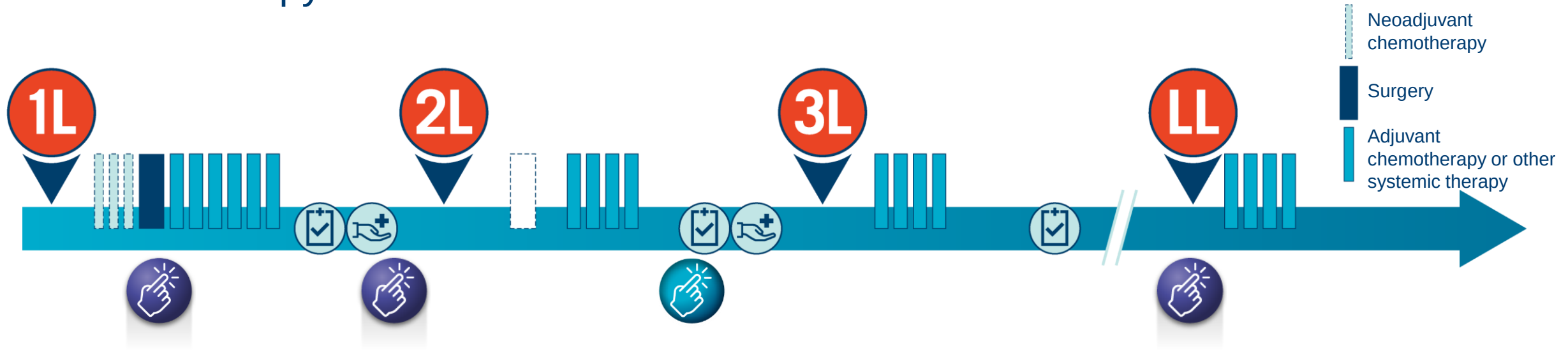
Responses to treatment and supportive care¹

An outcome related to treatment is called a treatment response. The four main possible treatment responses are:¹

- Complete response or complete remission: Tumor has responded to treatment and is no longer detectable by imaging or other tests.
- Partial response or partial remission: Tumor has responded to treatment and decreased in size but is still detectable.
- Stable disease or disease control: Tumor is not changing over time.
- Disease progression: Tumor has continued to grow or progress during or after treatment.

1. Shead DA et al. NCCN Guidelines®. Ovarian Cancer Epithelial Ovarian Cancer. V.1.2021. NCCN®, 2021:1-72.

Lines of Therapy



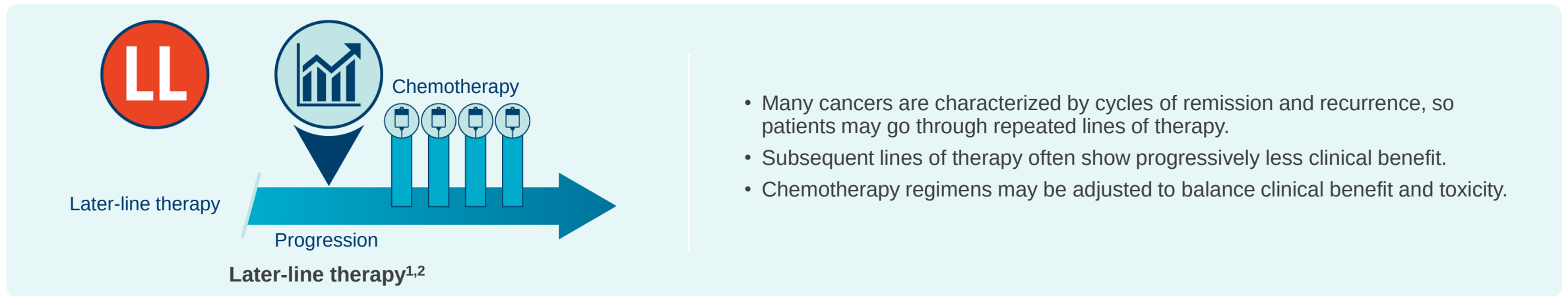
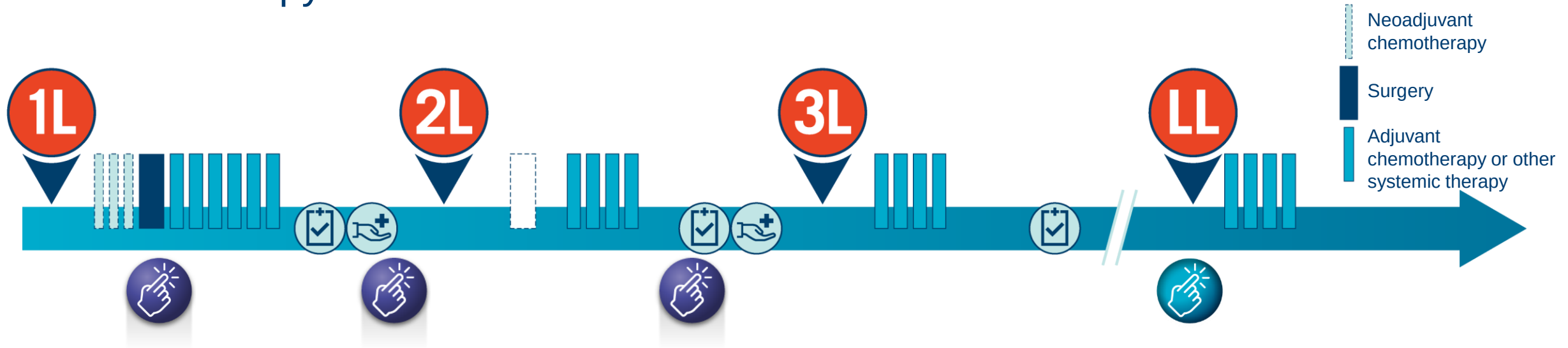
Second- and third-line therapies:^{1,2}

- Second- and third-line therapies are called subsequent therapy.
- Subsequent therapy can be initiated when a patient's prior line of therapy stops working and the disease is progressing.

1. Freidlin B et al. *J Natl Cancer Inst.* 2015;107:djv225.

2. NCI. *NCI Dictionary of Cancer Terms.* Bethesda, MD: NIH; 2020.

Lines of Therapy



1. ACS. *How Is Chemotherapy Used to Treat Cancer*. Atlanta, GA. November 2019.
 2. ACS. *Managing Cancer as a Chronic Illness*. Atlanta, GA. January 2019.

Maintenance Therapy

Patients who have stable disease or a response after therapy may be given maintenance therapy in addition to best supportive care. In maintenance therapy, patients receive additional therapy with the goal of preserving a lasting remission or prolonging the disease-free interval.^{1,2}



Maintenance therapy:^{2,3}

- Therapy with an agent typically given after completion of therapy to maintain or prolong benefits and delay the need for next-line therapy
- Can be given until disease progression or unacceptable toxicity, or for a fixed period of time



Click to learn about factors which may determine the use of maintenance therapy.

1. Freidlin B et al. *J Natl Cancer Inst.* 2015;107:djv225.
2. Grivas P et al. *Target Oncol.* 2019;14:505.
3. NCI. *NCI Dictionary of Cancer Terms.* Bethesda, MD: NIH; 2020.

Factors in Use of Maintenance Therapy



Tolerability, cost, and biomarker status represent the major factors for both patients and physicians in the choice of whether to apply maintenance therapy or the active surveillance approach, in which the patient is monitored but cancer treatment is not given until the disease becomes detectable.^{1,2,3}

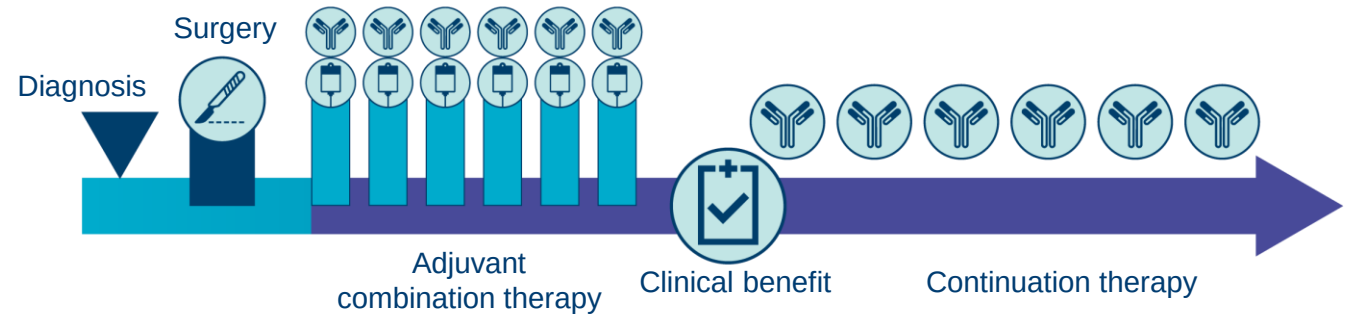
1. DiSilvestro P et al. *Cancer Treatment Reviews*. 2018;69:53.
2. NCI. *NCI Dictionary of Cancer Terms*. Bethesda, MD: NIH; 2020.
3. Shead DA et al. NCCN Guidelines®. Ovarian Cancer Epithelial Ovarian Cancer. V.1.2021. NCCN®. 2021:1-72.

Continuation vs Maintenance Therapy

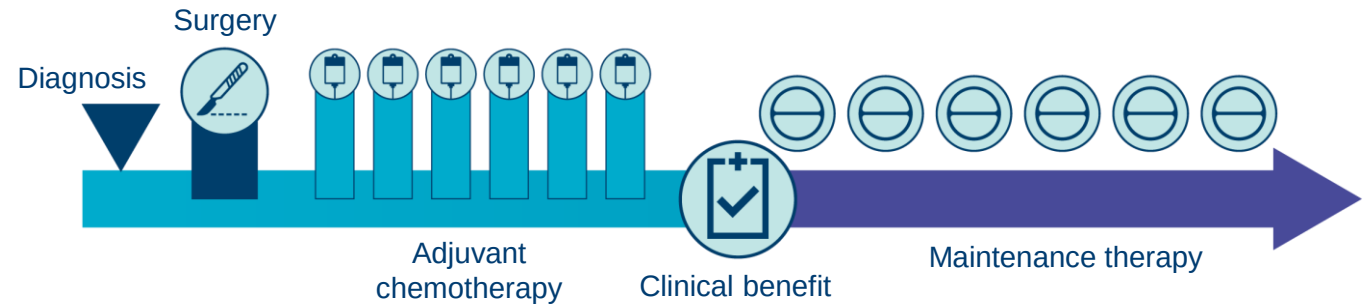
Maintenance therapy can be given as:



Continuation therapy: An agent given as part of the therapy regimen continues to be administered; also called continuation maintenance therapy.^{1,2}



Maintenance therapy: A new agent, not part of the therapy regimen, is administered; also called switch maintenance therapy.^{1,2}



Continuation and maintenance therapy^{1,2,3}

1. Grivas P et al. *Target Oncol.* 2019;14:505.
2. NCCN®. Patient and Caregiver Resources — Dictionary. Plymouth Meeting, PA; 2020.
3. Freidlin B et al. *J Natl Cancer Inst.* 2015;107:djv225.

Overview of Ovarian Cancer Treatment

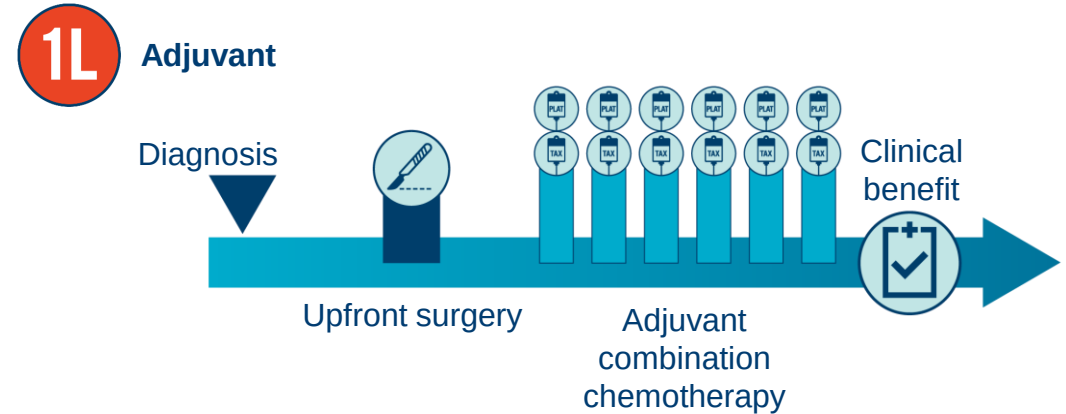
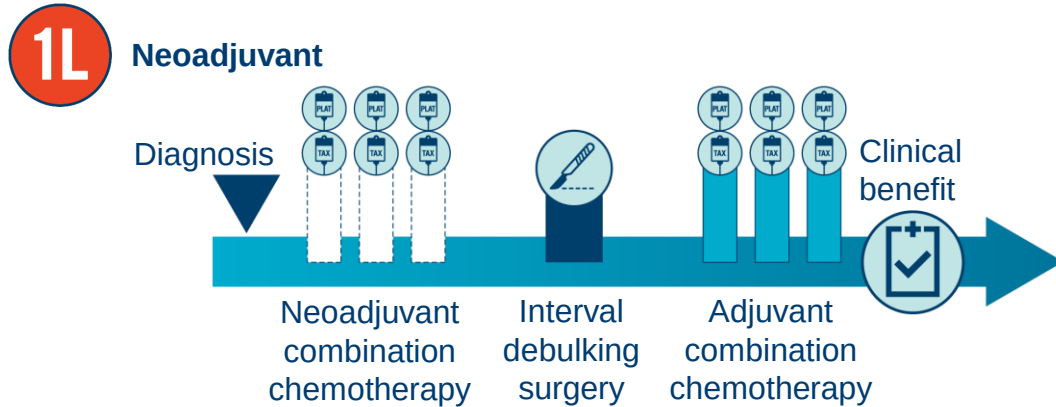


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First-Line Ovarian Cancer Therapy

First-line therapy in patients with advanced ovarian cancer typically involves surgery (to remove tumors) combined with neoadjuvant or adjuvant combination chemotherapy (to prevent recurrence).¹



First-line ovarian cancer therapy^{1,2}



Combination chemotherapy¹

- Platinum-based agent plus a taxane
- Administration of 3–4 cycles (neoadjuvant) or 6 cycles (adjuvant)



Surgery¹

- Referred to as debulking or cytoreduction
- Can be upfront (before chemotherapy) or interval (after chemotherapy)
- Goal is removal of all visible disease



Response rate: up to 80%²

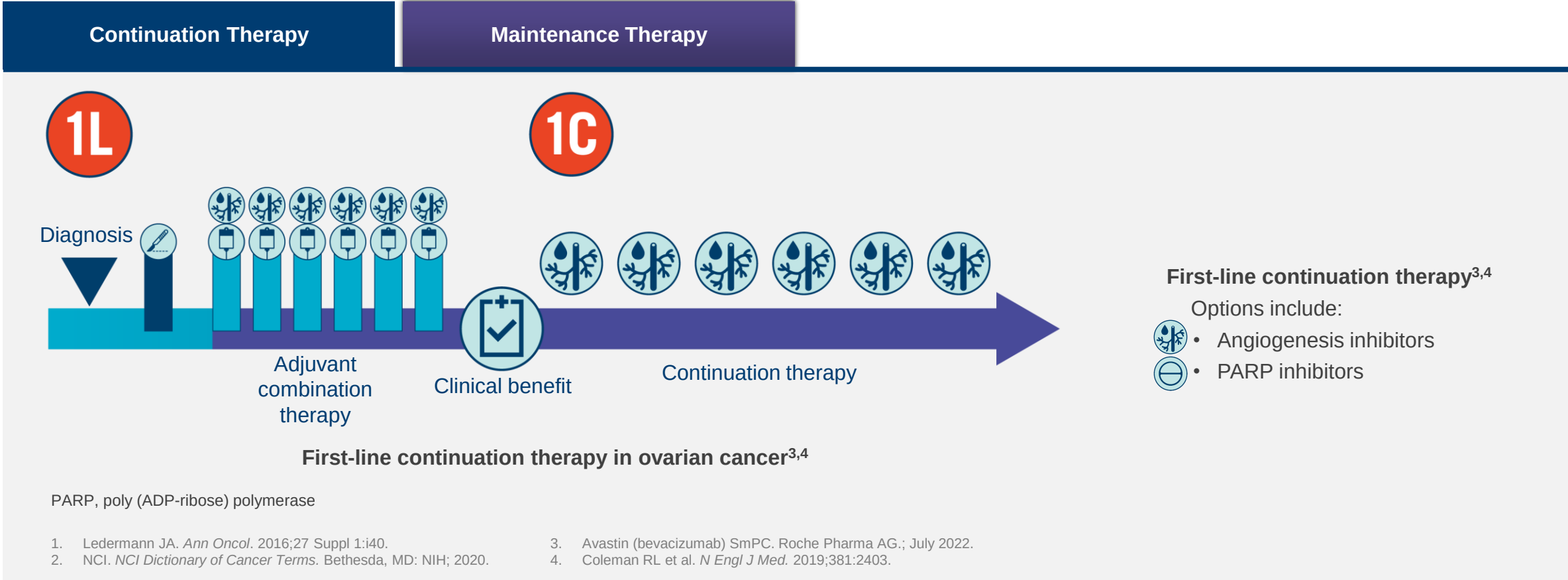
Complete response rate: 50%²

1. Ledermann JA et al. *Ann Oncol.* 2013;24 Suppl 6:vi24.

2. Rose PG, Uyar DS. Ovarian Cancer. In: Bieber EJ, Sanfilippo JS, Horowitz IR, et al, eds. *Clinical Gynecology*. 2nd ed. Cambridge, UK: Cambridge University Press; 2015:790-812.

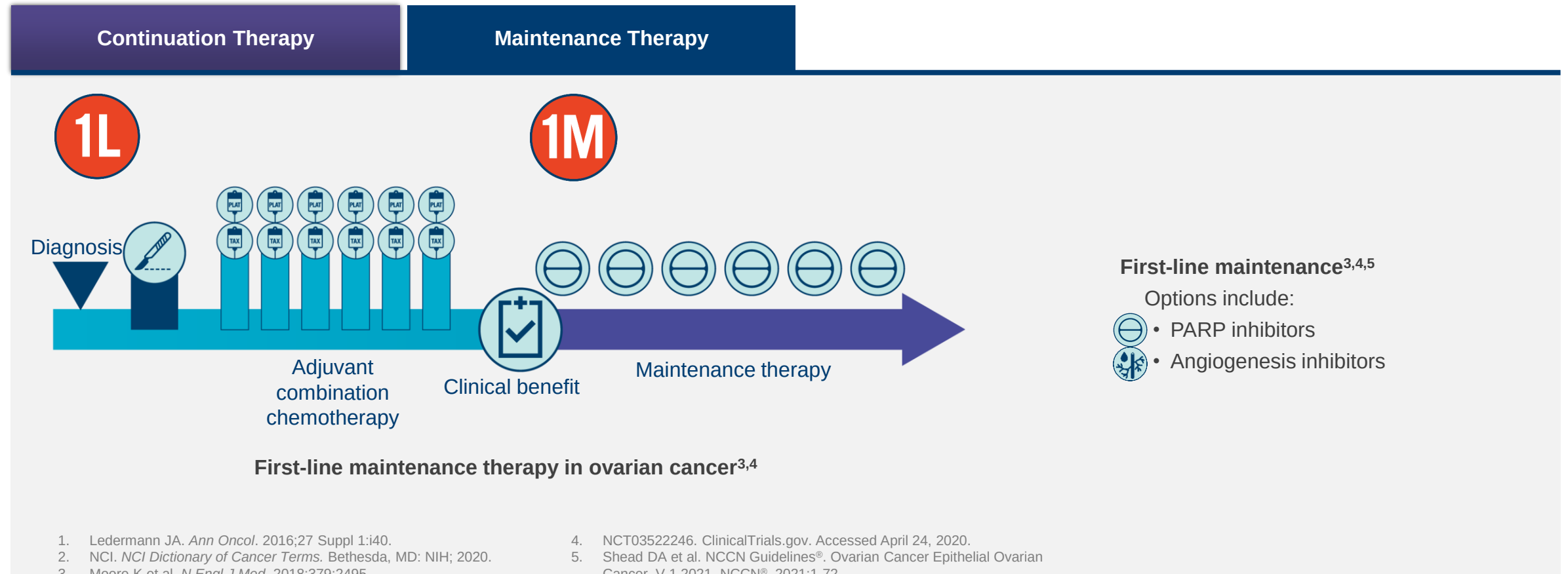
First-Line Maintenance Therapy

After first-line therapy for ovarian cancer, patients in response can receive maintenance or continuation therapy, with the goal of preventing or delaying disease recurrence or progression, thereby avoiding or delaying the need to initiate a new course of chemotherapy.^{1,2}



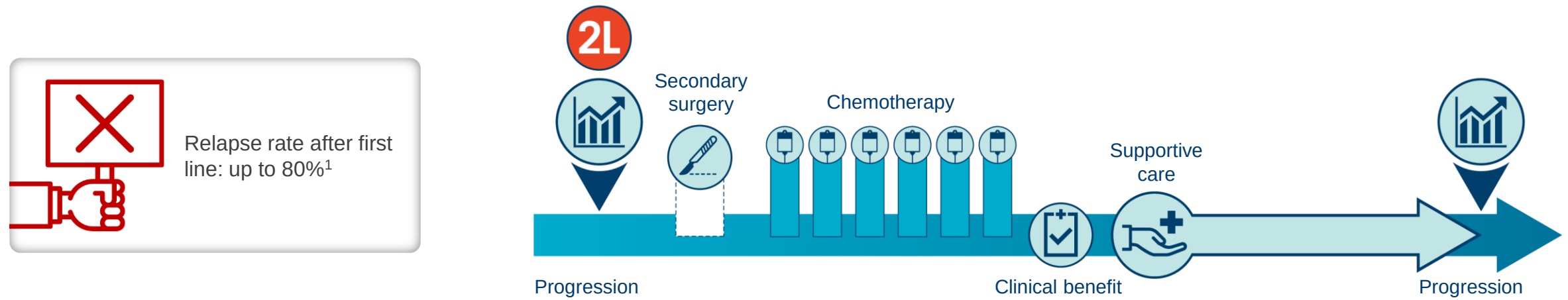
First-Line Maintenance Therapy

After first-line therapy for ovarian cancer, patients in response can receive maintenance or continuation therapy, with the goal of preventing or delaying disease recurrence or progression, thereby avoiding or delaying the need to initiate a new course of chemotherapy.^{1,2}



Recurrent Ovarian Cancer — Second-Line Therapy

About 80% of patients with advanced ovarian cancer will experience a relapse and can be treated with subsequent therapy. Recurrent ovarian cancer is considered incurable, but life-prolonging remissions can still be attained.^{1,2}



Second-line therapy for ovarian cancer^{2,3,4}



Surgery³

- Not standard in recurrent disease
- Only useful when tumor can be completely resected



Chemotherapy³

- Can be monotherapy or combination therapy, platinum-based or non-platinum-based
- Choice of therapy depends on response to prior therapies and presence of comorbidities caused by earlier therapy

1. Hanker LC et al. *Ann Oncol.* 2012;23:2605.

2. Rose PG, Uyar DS. Ovarian Cancer. In: Bieber EJ, Sanfilippo JS, Horowitz IR, et al, eds. *Clinical Gynecology*. 2nd ed. Cambridge, UK: Cambridge University Press; 2015:790-812.

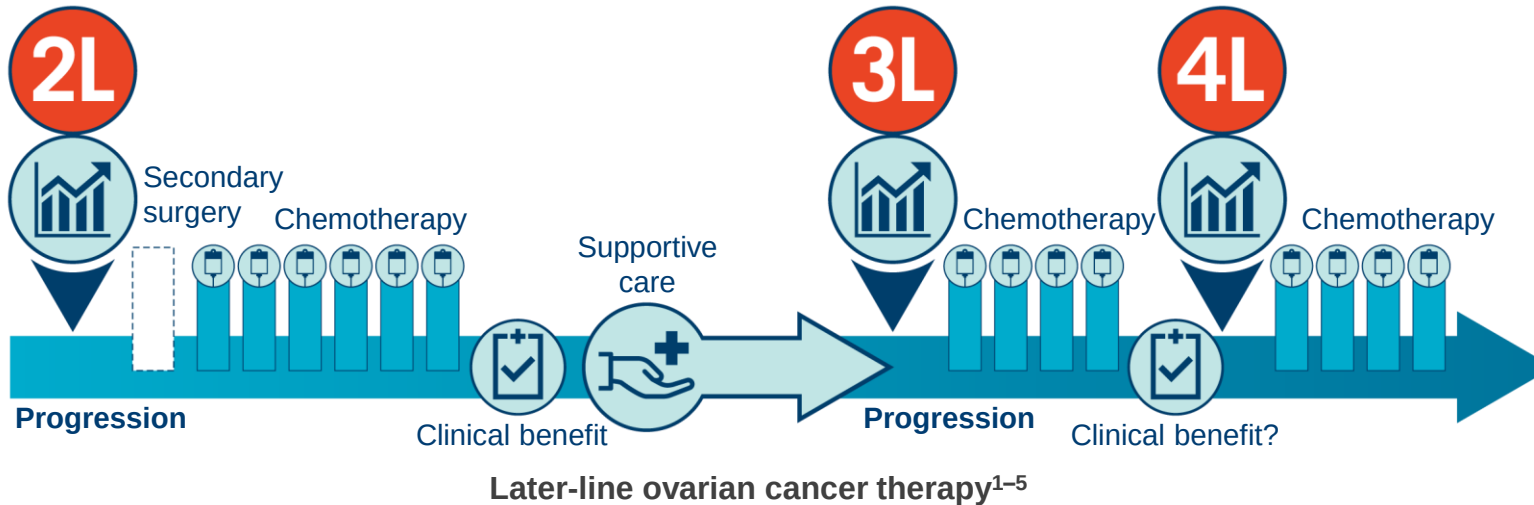
3. Ledermann JA et al. *Ann Oncol.* 2013;24 Suppl 6:vi24.

4. Parmar MK et al. *Lancet.* 2003;361:2099.

Recurrent Ovarian Cancer — Later Therapy

Ovarian cancer is characterized by cycles of remission and recurrence, so many patients receive multiple lines of therapy. Therefore, recurrence therapy must balance efficacy with minimization of toxicity and preservation of the patient's quality of life.^{1,2}

Later-line (third-line and later) therapies typically show progressively lower response rates, shorter times to progression, and shorter survival.^{2,3}

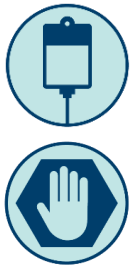


Response rates^{2,3}

- Second-line: 70%
- Fifth-line: 4.5%

Median survival (months) after^{2,3}

- Second relapse: 11.3
- Fifth relapse: 5.0



Later-line chemotherapy¹⁻⁵

- Later-line therapy is typically single-agent chemotherapy.
- Choice of therapy depends on response to prior therapies and presence of comorbidities caused by earlier treatment, patient **performance status**, and the preferences of the patient.
- Chemotherapy can be associated with cumulative or irreversible toxicities, such as myelosuppression, kidney toxicity, neurotoxicity, and cardiotoxicity. Toxicity must be taken into account before retreatment with chemotherapy in the later-line setting.

1. Rose PG, Uyar DS. Ovarian Cancer. In: Bieber EJ, Sanfilippo JS, Horowitz IR, et al, eds. *Clinical Gynecology*. 2nd ed. Cambridge, UK: Cambridge University Press; 2015:790-812.
2. Hanks LC et al. *Ann Oncol*. 2012;23:2605.

3. Bruchim I et al. *Eur J Obstet Gynecol Reprod Biol*. 2013;166:94.
4. Fotopoulou C. *EJC Suppl*. 2014;12:13.
5. Parmar et al. *Lancet*. 2003;361:2099.

Glossary



Performance status

Standardized criteria for measuring how a disease impacts a patient's abilities and activities of daily living (walking, working, etc).¹

1. Eastern Cooperative Oncology Group (ECOG). *ECOG Performance Status*. Philadelphia, PA: 2020.

Other Recurrent Ovarian Cancer Treatments

Besides chemotherapy, other classes of systemic therapy can be used in recurrent disease:^{1,2}



Immune checkpoint inhibitors



Anti-angiogenesis drugs



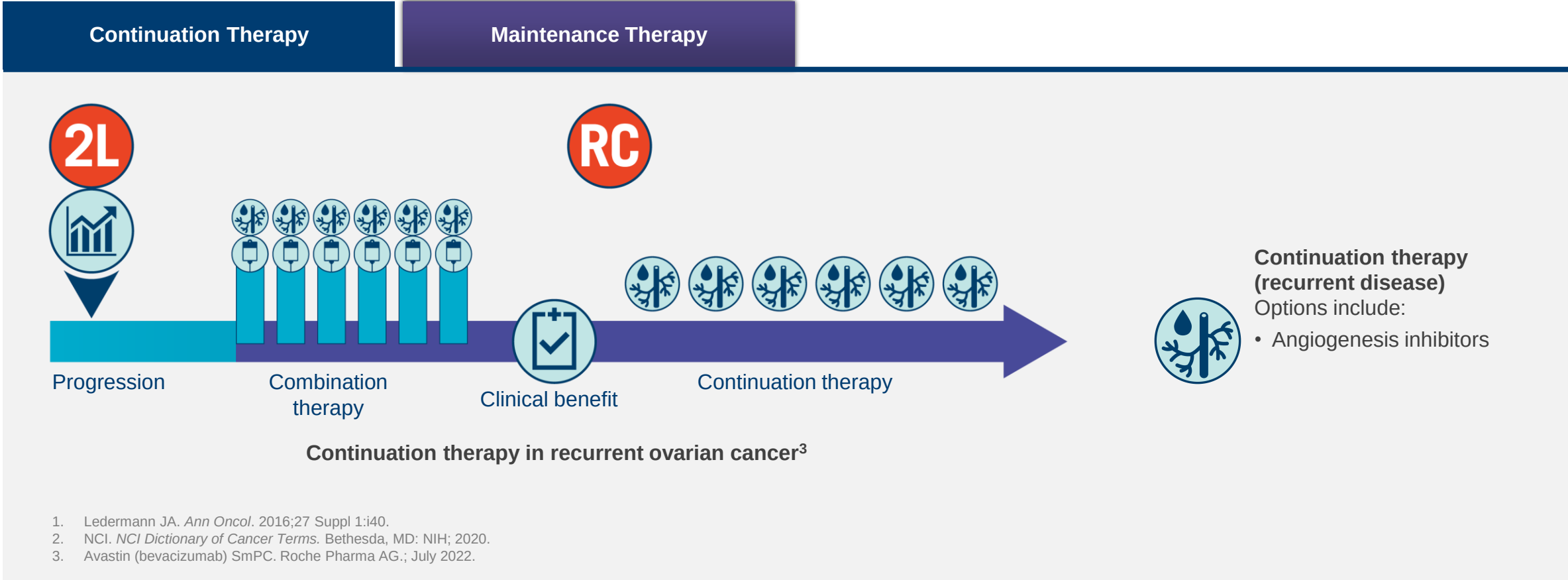
Several combinations are currently being tested, including PARP inhibitors with angiogenesis inhibitors, PARP inhibitors with checkpoint inhibitors, immune checkpoint inhibitors with angiogenesis inhibitors, or a combination of all three.¹

PARP, poly (ADP-ribose) polymerase

1. Naumann et al. *Gynecol Oncol.* 2019;153:436.
2. Lynparza® (olaparib) PI. AstraZeneca Pharmaceuticals LP; August 2022.

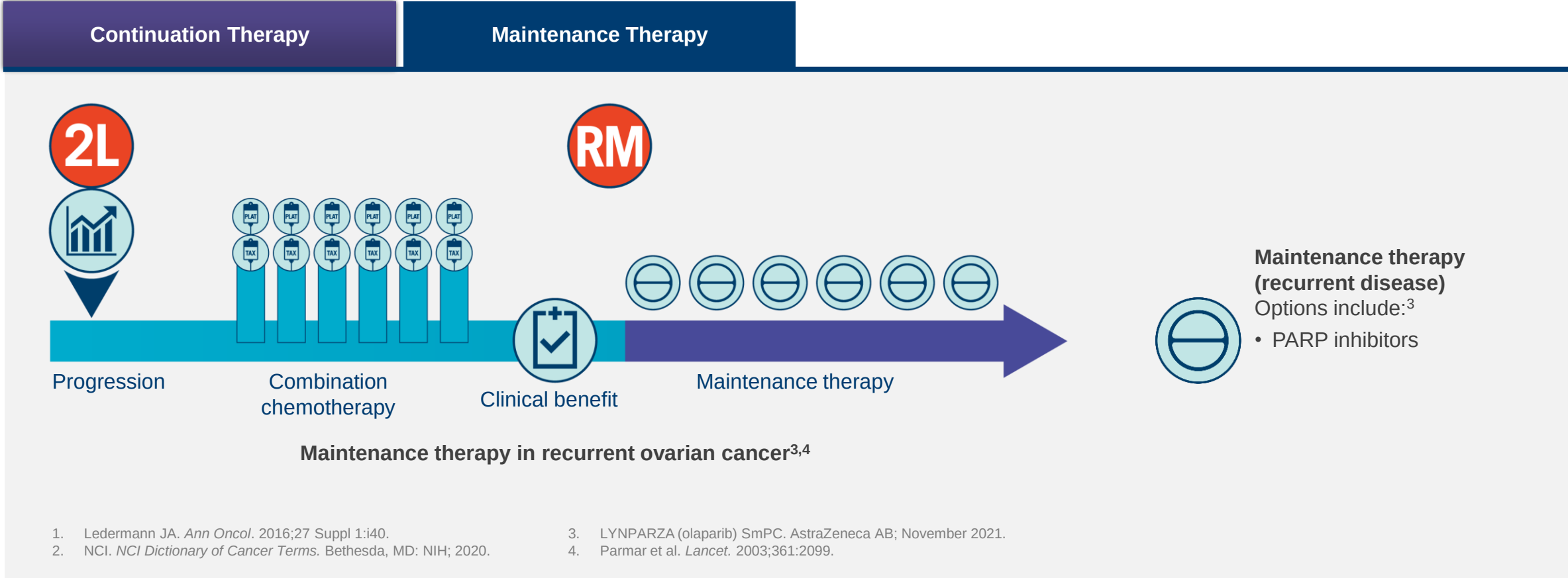
Maintenance Therapy in Recurrent Disease

Patients with recurrent disease in response to chemotherapy (second-line or later-line) can receive maintenance therapy, with the goal of preventing or delaying disease recurrence or progression, thereby prolonging the disease-free interval and delaying the need to initiate a new course of therapy.^{1,2}



Maintenance Therapy in Recurrent Disease

Patients with recurrent disease in response to chemotherapy (second-line or later-line) can receive maintenance therapy, with the goal of preventing or delaying disease recurrence or progression, thereby prolonging the disease-free interval and delaying the need to initiate a new course of therapy.^{1,2}

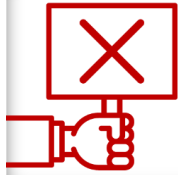


Unmet Needs in Ovarian Cancer



Response rate: up to 80%¹
Complete response rate: 50%¹

- 20% of patients do not respond to first-line surgery plus chemotherapy, and half do not achieve a complete response.¹
- Novel first-line therapies are needed to optimize complete response rate.¹



Relapse rate after first-line:
up to 80%^{2,3}

- The majority of patients who respond to first-line therapy will eventually relapse.^{2,3}
- Maintenance therapy options in the first-line and recurrent disease settings are required to prevent or delay disease recurrence or relapse.^{2,3}



Median survival (months) after^{2,4}

- Second relapse: 11.3
- Fifth relapse: 5.0

- Later-line (third-line and later) therapies typically show progressively lower response rates, shorter times to progression, and shorter survival.^{2,4}
- More effective later-line treatment options are required.^{2,4}



- Chemotherapy can be associated with cumulative or irreversible toxicity.⁵
- Treatment options with more acceptable toxicity profiles are required, particularly as later-line therapy.⁵

1. Rose PG, Uyar DS. Ovarian Cancer. In: Bieber EJ, Sanfilippo JS, Horowitz IR, et al, eds. *Clinical Gynecology*. 2nd ed. Cambridge, UK: Cambridge University Press; 2015:790-812.
2. Harker LC et al. *Ann Oncol*. 2012;23:2605.

3. Ledermann JA. *Ann Oncol*. 2016;27 Suppl 1:i40.
4. Bruchim I et al. *Eur J Obstet Gynecol Reprod Biol*. 2013;166:94.
5. Fotopoulou C. *EJC Suppl*. 2014;12:13-6.

Post-Treatment Care

When primary treatment is completed, patients will be monitored by their oncologist for several years. This is important because most treated patients with advanced ovarian cancer will relapse. Survivors of ovarian cancer also have an increased risk of developing other types of cancer.^{1,2}

Follow-up

Recurrence vs second cancer

Follow-up usually includes:



Careful clinical examination, including a physical exam and a pelvic exam¹



Blood tests for tumor markers:
Most common marker for epithelial ovarian cancer is CA-125, which is elevated in many women with advanced ovarian cancer¹



Imaging tests that include CT scans, PET scans, or MRI, depending on any symptoms or other concerning signs¹



The ESMO-ESGO guidelines recommend when following up, a patient should be assessed every 3–4 months for the first 2 years, and every 6 months during years 3 to 5³

CA-125, cancer antigen 125; CT, computed tomography; ESGO, European Society of Gynecological Oncology; ESMO, European Society for Medical Oncology; MRI, magnetic resonance imaging; PET, positron emission tomography.

1. ACS. *Living as an Ovarian Cancer Survivor*. Atlanta, GA: April 2018:1-3.
2. Harker LC et al. *Annals of Oncology*. 2012;23:2605.
3. Colombo N et al. *Annals of Oncology*. 2019;30:672.

Post-Treatment Care

When primary treatment is completed, patients will be monitored by their oncologist for several years. This is important because most treated patients with advanced ovarian cancer will relapse. Survivors of ovarian cancer also have an increased risk of developing other types of cancer.^{1,2}

Follow-up

Recurrence vs second cancer



If a cancer does come back after treatment, it is called recurrence. Some cancer survivors may develop a new unrelated cancer later; this is called a second cancer.³

Second cancers can be caused by the treatment given for the first cancer (eg, leukemia caused by platinum-based agents).³

Survivors of ovarian cancer can get any type of second cancer, but they have an increased risk of:³

- Breast cancer
- Bladder cancer
- Bile duct cancer
- Melanoma of the eye
- Acute leukemia
- Colon cancer
- Rectal cancer
- Small intestine cancer
- Cancer of the renal pelvis



Genetic factors that may have contributed to the development of ovarian cancer can add to the risk of developing other cancers.³

1. ACS. *Living as an Ovarian Cancer Survivor*. Atlanta, GA: April 2018:1-3.
2. Hanker LC et al. *Annals of Oncology*. 2012;23:2605.

3. ACS. *Second Cancers After Ovarian Cancer*. Atlanta, GA: April 2018:1-2.

Summary

Treatment Principles

- Patients newly diagnosed with cancer typically receive an initial, fixed-duration, standard set of treatments called first-line therapy; when the first treatment stops working and the cancer progresses, a second set of treatments (second-line therapy or first recurrence therapy) can be initiated.
- Chemotherapy is a form of systemic treatment for cancer, using drugs that stop tumor growth either by killing cancer cells or stopping them from dividing.
- Neoadjuvant therapy, such as chemo- or radiotherapy, may be given as a first step to shrink a tumor before surgery.
- Adjuvant therapy is additional cancer treatment, such as chemotherapy or radiotherapy, given after the primary treatment to lower the risk that the cancer will come back.
- The four main possible treatment responses are complete remission, partial remission, stable disease, or disease progression.
- Maintenance therapy is treatment with an agent typically given after completion of therapy to maintain or prolong the benefits of therapy and delay the need for next-line therapy.
- Maintenance therapy can be given as:
 - Continuation therapy, in which an agent given as part of the therapy regimen continues to be administered, or
 - Maintenance therapy, in which a new agent, not part of the therapy regimen, is administered

Overview of Ovarian Cancer Treatment

- First-line therapy in patients with advanced ovarian cancer typically involves surgery combined with neoadjuvant or adjuvant combination chemotherapy.
- About 80% of patients with advanced-stage ovarian cancer will experience a relapse, and can be treated with additional lines of therapy, usually involving chemotherapy but in some cases surgery as well.
- Tolerability, cost, and biomarker status represent the major factors for both patients and physicians in the choice of whether to apply maintenance therapy or the active surveillance approach.
- After the end of primary treatment, patients will be followed by their oncologist for several years.
- A follow-up usually includes careful clinical examination, blood tests, and imaging tests.
- ESMO guidelines recommend a follow-up frequency of every 3 to 4 months during the first 2 years, and every 6 months during years 3 through 5.
- Some cancer survivors may develop a new, unrelated cancer later: this is called a second cancer.
- Survivors of ovarian cancer can get any type of second cancer, but they have an increased risk of breast, bladder, and bile duct cancer, as well as melanoma of the eye and acute leukemia.

End of Module

You have reached the end of the Treatment Principles module.



Congratulations!

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